

Patient Name : Vinod Malhotra
Age / Sex : 81 Y / M
Referred By : SELF
Centre : BTC CROSSINGS REPUBLIK

Lab No : CRG26011420
Registration On : 29-Jan-26 08:56
Patient ID : UHOM.0000075200

Urine R/M				Urine Sample
Accession No: CP00845606	Collected On: 29-Jan-26 08:56	Received On: 29-Jan-26 12:38	Approved On: 29-Jan-26 13:52	
Observation	Result	Unit	Biological Ref. Interval	Method
Physical Examination				
Urine Quantity	7.5	mL	7 - 8	Physical Examination
Urine Colour	Yellow		Pale Yellow	Physical Examination
Urinary Transparency	Clear		Clear	Physical Examination
Biochemical Examination				
Urinary pH	6.0	pH	6.0 - 8.0 pH	bromothymol blue
Urinary Specific Gravity	1.020		1.005 - 1.030	Ethyleneglycol-bis t.a.a.
Urinary Protein	Negative		Negative	Tetrachlorophenol
Urinary Glucose	Negative		Negative	glucose-oxidase-peroxidase
Urinary Ketones	Negative		Negative	Sodium Nitroprusside
Urobilinogen	Normal		Normal	Methoxybenzene Diazonium
Urine Bilirubin	Negative		Negative	Dichlorobenzene-diazonium
Urinary Nitrites	Negative		Negative	hydroxy
Blood [In Urine]	Negative		Negative	Tetramethylbenzidine
Leukocyte esterase	Negative		Negative	indoxyl-ester-diazonium
Microscopic Examination				
Pus Cells [In Urine]	1-2	/HPF	1 - 2 /HPF	Flow Micro Imaging
Epithelial Cells (Squamous)	1-2	/HPF	0-2/HPF	Flow Micro Imaging
Epithelial Cells (Non-Squamous)	NIL	/HPF	0-2/HPF	Flow Micro Imaging
Urinary RBC	NIL	/HPF	NIL /HPF	Flow Micro Imaging
Hyaline Casts	NIL	/LPF	0-2/LPF	Flow Micro Imaging
Pathological Casts	NIL	/LPF	0-1/LPF	Flow Micro Imaging
Yeast Cells	NIL	/HPF	0-1/HPF	Flow Micro Imaging
Crystals	NIL	/HPF	NIL/HPF	Flow Micro Imaging
Other Morphology	NIL		NIL	Microscopy

Remarks:

- Sample Quantity is observed after transfer to a Urinalysis Vacutainer Tube for preservation of sample.
- **Note for Female Patients:** If the urine is collected during menstruation, red cells may be present in the urine.
- **Microscopy:** Microscopy may have supplemented automated measurements, wherever necessary.

Advise: Please correlate results clinically.

Amita Gupta
Dr. Amita Gupta
 Consultant – Pathologist
 M.B.B.S., M.D. (Pathology)
 DMC Reg. No.: 91678

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CBC			EDTA Whole Blood Sample	
Accession No: 0317430EDT		Collected On: 29-Jan-26 08:56	Received On: 29-Jan-26 12:10	Approved On: 29-Jan-26 13:22
Observation	Result	Unit	Biological Ref. Interval	Method
Hemoglobin	10.9	gm/dL	13.0 - 17.0	Colorimetric
Total RBC	5.60	million/ μ L	4.5 - 5.5	Electrical Impedance
Platelet Count	189	$\times 10^3 / \mu$ L	150 - 410 $\times 10^3 / \mu$ L	Impedance
Total Leucocyte Count (WBC)	8.55	$\times 10^3 / \mu$ L	4.0 - 10.0	Flow Cytometry
Differential Leucocyte Count (DLC)				
Neutrophils	69.4	%	40 - 80	Flow Cytometry
Lymphocytes	20.8	%	20 - 40	Flow Cytometry
Monocytes	6.8	%	2 - 10	Flow Cytometry
Eosinophils	2.5	%	1 - 6	Flow Cytometry
Basophils	0.5	%	0 - 1	Flow Cytometry
Absolute Neutrophil Count	5.93	$\times 10^3 / \mu$ L	2.0 - 7.5	Flow Cytometry
Absolute Lymphocyte Count	1.78	$\times 10^3 / \mu$ L	1.0 - 4.0	Flow Cytometry
Absolute Monocyte Count	0.58	$\times 10^3 / \mu$ L	0.2 - 1.0	Flow Cytometry
Absolute Eosinophil Count	0.21	$\times 10^3 / \mu$ L	0.02 - 0.5	Flow Cytometry
Absolute Basophil Count	0.05	$\times 10^3 / \mu$ L	0.00 - 0.30	Flow Cytometry
Indices				
Hematocrit (PCV)	34.9	%	40 - 50	Calculated
Mean Corpuscular Volume (MCV)	62.4	fL	83 - 101	Calculated
Mean Corp. Hemoglobin (MCH)	19.4	pg	27 - 32	Calculated
MCH Concentration (MCHC)	31.2	g/dl	31.5 - 34.5	Calculated
Red Cell Dist. Width (RDW-CV)	17.6	%	11.5 - 14.5	Calculated
Red Cell Dist. Width (RDW-SD)	46.4	fL	39 - 46	Calculated
Mean Platelet Volume (MPV)	14.2	fL	7-5 - 12.0	Calculated
Mentzer Index	11.14	Index		Calculated

Remarks: Please correlate with clinical conditions



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Glucose Fasting			Sodium Fluoride Sample	
Accession No: 0340990GLU		Collected On: 29-Jan-26 08:56	Received On: 29-Jan-26 12:16	Approved On: 29-Jan-26 17:18
Observation	Result	Unit	Biological Ref. Interval	Method
Blood Sugar Fasting	114	mg/dL	70 - 100	GOD/POD, colorimetric

Sample Type: Sodium Fluoride; A blood sample will be taken after 8 - 12 hours of fasting.

Method: Glucose oxidase hydrogen peroxidase

Technology: Dry Chemistry (VITROS MicroSlide, MicroSensor & Intellicheck Technology)

Analyzer: Fully Automated Integrated Biochemistry & ImmunoAssay Analyzer: VITROS 5600

Remarks: Please correlate clinically

Note: Blood glucose level is maintained by a very complex integrated mechanism involving a critical interplay of the release of hormones and action of enzymes on key metabolic pathways. If postprandial glucose is lower than fasting glucose, it is termed as postprandial reactive hypoglycemia (PRH). The possible cause of PRH are high insulin sensitivity, exaggerated response of insulin and glucagon-like peptide 1, defects in counter-regulation, very lean individuals, anxious individuals, after massive weight reduction, women with

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lower body overweight physical activity prior test, hypoglycemic medication, deliberately eating less or eat a non-carbohydrate meal before testing.

Kidney Function Test				Serum Sample
Accession No: 0346832SST	Collected On: 29-Jan-26 08:56	Received On: 29-Jan-26 12:21	Approved On: 29-Jan-26 14:20	
Observation	Result	Unit	Biological Ref. Interval	Method
Blood Urea	36	mg/dL	19 - 43	Urease, Colorimetric
Blood Urea Nitrogen	16.82	mg/dL	9-20	Calculated
Creatinine	1.07	mg/dL	0.6-1.25	Enzymatic
Estimated GFR	69.70	mL/min/1.73m2		Calculated By CKD-EPI(2021)
Uric Acid	5.3	mg/dL	3.5 - 8.5	Uricase , Colorimetric
Calcium	9.5	mg/dL	8.4 - 10.2	Arsenazo III
Phosphorus	4.6	mg/dL	2.5 - 4.5	Phosphomolybdate reduction
BUN/Creatinine Ratio	15.72	Ratio		Calculated
Urea/Creatinine Ratio	33.64	Ratio		Calculated
Electrolytes				
Sodium	139	mmol/L	137-145	ISE Direct
Potassium	4.9	mmol/L	3.5 - 5.1	ISE Direct
Chloride	105	mmol/L	98 - 107	ISE Direct

Classification of eGFR by UK Kidney Association (2017):		
eGFR (ml/min/1.73 m2)	GFR Category	Significance
>90	G1	Normal Renal Function
60-90	G2	Mild Impairment of Renal Function
45-59	G3a	Impaired Kidney Function
30-44	G3b	Impaired Kidney Function
15-29	G4	Significant Impairment of Renal Function
<15	G5	End-Stage Renal Failure (ESRF)

Technology: Dry Chemistry (VITROS MicroSlide, MicroSensor and Intellitect Technology)
Remarks: Please correlate results clinically.

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Hb A1c				EDTA Whole Blood Sample
Accession No: 0317430EDT	Collected On: 29-Jan-26 08:56	Received On: 29-Jan-26 12:10	Approved On: 29-Jan-26 14:56	
Observation	Result	Unit	Biological Ref. Interval	Method
HbA1C	6.8	%	4.8-5.7	HPLC
90 Day Average Blood Glucose	148.46	mg/dl	90 - 120	Calculated

Interpretation as per American Diabetes Association (ADA) Guidelines

Reference Group	Non diabetic adults >=18 years	At risk (Prediabetes)	Diagnosing Diabetes	Therapeutic goals or glycemic control
HbA1c in %	4.0-5.6	5.7-6.4	>= 6.5	<7.0

Note:

- Presence of Hemoglobin variant and /or conditions that affect red cell turnover must be considered particularly when the HbA1c result does not correlate with the patient's blood glucose levels.
- Factors That Interfere With HbA1c Measurement:** Hemoglobin variants, elevated fetal hemoglobin (HbF) and chemically modified derivatives of hemoglobin (e.g. carbamylated Hb in patients with renal failure) can affect the accuracy of HbA1c measurements
- Factors That Affect Interpretation Of HbA1c Results:** Any condition that shortens erythrocyte survival or decreases mean erythrocyte age (e.g., recovery from acute blood loss, hemolytic anemia, Hbss, HbCC, and HbSC) will falsely lower HbA1c test results regardless of the assay method used. Iron deficiency anemia is associated with higher HbA1c

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- Since HbA1c Reflects long term Fluctuations in the blood glucose concentration, a diabetic patient who is recently under good control may still have a high concentration of HbA1c. Converse is true for a diabetic previously under good control but now poorly controlled.
- Target goals of < 7.0% may be beneficial in patients with short duration of diabetes, long life expectancy and no significant cardiovascular co-morbid conditions, targeting a goal of > 7.0 % may not be appropriate.
- Any condition that shortens erythrocyte survival such as sickle cell disease, pregnancy (second and third trimesters), hemodialysis, recent blood loss of transfusion , or erythropoietin will falsely lower HbA1c results regardless of the assay method
- In patients with HbA1c level between 7-8%, Glycemark (1,5 Anhydro-glucitol) test may be done to identify those with more frequent and extreme hyperglycemic excursions.

Remarks: Please correlate results with clinical conditions.

Ruhani Kanwar

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Liver Function Test				Serum Sample
Accession No: 0346832SST	Collected On: 29-Jan-26 08:56	Received On: 29-Jan-26 12:21	Approved On: 29-Jan-26 14:20	
Observation	Result	Unit	Biological Ref. Interval	Method
Total Protein	6.7	g/dL	6.5-7.8	Biuret, No Serum Blank
Albumin	3.8	g/dL	4.2 - 5.0	Bromocresol Green
Globulin	2.9	gm/dL	2.0-3.5	Calculated
A/G Ratio	1.31	Ratio	1.5-2.5	Calculated
Total Bilirubin	0.42	mg/dL	0.2-1.3	Azobilirubin/dyphylline
Conjugated Bilirubin	0.23	mg/dL	<0.3	Calculated
Unconjugated Bilirubin	0.19	mg/dL	<1.1	Spectrophotometry
SGOT (AST)	22	U/L	18-39	Enzymatic Colorimetric
SGPT (ALT)	13	U/L	4-50	UV with P5P
SGOT/SGPT Ratio	1.69	Ratio		Calculated
Alkaline Phosphatase	92	U/L	50 - 116	PNPP, AMP buffer
Gamma Glutamyl Transferase	16	U/L	13 - 109	G-glutamyl-p-nitroanilide

Clinical Significance of LFT: Clinical suspicion of liver disease usually leads to the measurement of the liver function tests (LFT) which include measurement of several enzymes, serum bilirubin and albumin. These parameters may point to an underlying pathological process and direct further investigation. The aim of investigation in patients with suspected liver disease are: · To detect hepatic abnormality · Measurement of severity of liver damage · Identify the specific cause · Investigate possible complications

Technology: Dry Chemistry (VITROS MicroSlide, MicroSensor and Intellicheck Technology)
Analyzer: Fully Automated Biochemistry and ImmunoAssay Analyzer: VITROS 5600

Advise: Please correlate results clinically.

Vitamin D, 25 - Hydroxy				Serum Sample
Accession No: 0346832SST	Collected On: 29-Jan-26 08:56	Received On: 29-Jan-26 12:21	Approved On: 29-Jan-26 15:01	
Observation	Result	Unit	Biological Ref. Interval	Method
25-OH Vitamin D (Total)	62.6	ng/mL	20 - 100	CLIA

Biological Reference Interval:

Range	Vitamin D Total, 25(OH)
Deficiency	<20 ng/mL
Insufficiency	20-29 ng/mL
Sufficiency	30-100 ng/mL

Technology: VITROS Microwell, Microsensor, and Intellicheck Technology
Analyzer: Fully Automated Integrated Biochemistry and ImmunoAssay: VITROS 5600

Clinical Significance: The major circulating form of vitamin D is 25-hydroxyvitamin D (25(OH)D); thus, the total serum 25(OH)D level is currently considered the best indicator of vitamin D supply to the body from cutaneous synthesis and nutritional intake. There are two principal forms of vitamin D: D2 and D3. Many of the currently available assays measure and report on both vitamin D2 and D3 metabolites. This can be useful in studies evaluating the contribution of vitamin D2 and D3 to overall vitamin D status. One exception is that 25(OH)D levels do not indicate clinical vitamin D status in patients with chronic renal failure or type 1 vitamin D-dependent rickets or when calcitriol (1,25-dihydroxy

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vitamin D) is used as a supplement.

Remarks: Please correlate results clinically.

Lipid Profile				Serum Sample
Accession No: 0346832SST	Collected On: 29-Jan-26 08:56	Received On: 29-Jan-26 12:21	Approved On: 29-Jan-26 14:20	
Observation	Result	Unit	Biological Ref. Interval	Method
Total Cholesterol	154	mg/dL	<200	Enzymatic (CHE/CHO/POD)
Triglyceride	130	mg/dL	<150	Enzymatic, Endpoint
HDL Cholesterol	35	mg/dL	>45	Direct Measure, PTA / MgCl ₂
VLDL Cholesterol	26	mg/dL	5-40	Calculated
LDL Cholesterol	93	mg/dL	<100	Friedewald Formula (Calculated)
Non-HDL Cholesterol	119	mg/dL	<130	Calculated
LDL / HDL Ratio	2.66	Ratio	1.5-3.5	Calculated
TC / HDL Ratio	4.4	Ratio	3-5	Calculated

Clinical Decision Limits*	Optimal	Above Optimal	Borderline High	High	Very High
Triglycerides	<150	-	150-199	200-499	>=500
Total Cholesterol	<200	200-239	-	>=239	-
LDL Cholesterol	<100	100-129	130-159	160-189	>=189
HDL Cholesterol	>45	-	40-45	<40	-
Non HDL Cholesterol**	<130	130 - 159	160 - 189	190 - 219	>=220

* Clinical Decision Limits are suggested from Tietz Fundamentals Of Clinical Chemistry And Molecular Diagnostics 8th Edition

** Suggested from National Lipid Association Recommendations for Patient Centered Management of Dyslipidemia: Part 1—Full Report (Volume 9, Issue 2, P129-169, March 01, 2015, Terry A. Jacobson, MD et al.

Analyzer: Fully Automated Integrated Biochemistry and ImmunoAssay Analyzer: VITROS 5600
Technology: Dry Chemistry (VITROS MicroSlide, MicroSensor & Intellitect Technology)

Reports of Lipid Profile are best obtained with 10 hours fasting.

Clinical Significance:

- Triglyceride: Very high levels of Triglyceride can be indicative of a significantly higher risk of coronary vascular disease. Elevation of triglyceride can be seen with fasting less than 12 hours, obesity medication, alcohol intake, diabetes mellitus or pancreatitis.
- Total Cholesterol: its fractions and triglycerides are the important plasma lipids identifying cardiovascular risk factor and in the management of cardiovascular disease. Values above 220 mg/dl are associated with increased risk of CHD regardless of HDL & LDL value.
- HDL - Cholesterol: Low levels of HDL are associated with an increased risk of coronary vascular disease even in the face of desirable levels of Cholesterol and LDL-Cholesterol.
- LDL - Cholesterol: levels can be strikingly altered by thyroid, renal and liver disease as well as hereditary factors. In case Triglyceride levels are more than 400 mg/dl, the patient is advised for a direct-LDL Cholesterol test.

Remarks: Please correlate results clinically.

Disclaimer: *Estimation of LDL cholesterol is done by direct method when the triglyceride is >400 mg/dl.

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ESR				EDTA Whole Blood Sample
Accession No: 0317430EDT	Collected On: 29-Jan-26 08:56	Received On: 29-Jan-26 12:10	Approved On: 29-Jan-26 13:07	
Observation	Result	Unit	Biological Ref. Interval	Method
ESR	31	mm/hr	<20	Modified Westergren

Clinical Notes for ESR:

Increased Levels are commonly observed in:

- In any chronic infection
- Active rheumatic fever
- Acute myocardial infection
- Nephrosis
- All type of shocks

Decreased Levels are commonly observed in:

- Newborn infants

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- Polycythemia
 - Congestive heart failure
 - Sickel cell anaemia

Remarks: Please correlate results with clinical conditions.



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Vitamin B12				Serum Sample
Accession No: 0346832SST	Collected On: 29-Jan-26 08:56	Received On: 29-Jan-26 12:21	Approved On: 29-Jan-26 15:31	
Observation	Result	Unit	Biological Ref. Interval	Method
Vitamin B-12	371	pg/mL	239-931	CLIA

Technology: VITROS Microwell, Microsensor, and Intellicheck Technology
Analyzer: Fully Automated Integrated Biochemistry and ImmunoAssay: VITROS 5600

Remarks: Please correlate results clinically.



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Free Thyroid Test [FT3,FT4,TSH]				Serum Sample
Accession No: 0346832SST	Collected On: 29-Jan-26 08:56	Received On: 29-Jan-26 12:21	Approved On: 29-Jan-26 15:14	
Observation	Result	Unit	Biological Ref. Interval	Method
Free Triiodothyronine [FT3]	3.3	pg/mL	2.77 - 5.27	CLIA
Free Thyroxine [FT4]	0.92	ng/dL	0.78 - 2.19	CLIA
Thyroid Stimulating Hormone (TSH)	5.5	mIU/L	0.46-4.68	CLIA

Note:

- TSH Levels are subject to circadian variation**, reaching peak levels between 2-4 AM & the minimum between 6-10 PM. The variation is of the order of 50-206% Hence time of the day has influence on the measured serum TSH concentrations (Reference:Tietz Textbook of Clinical Chemistry & Molecular Diagnostics - 5th Edition Page 123). Fluctuating TSH value must be Clinically correlated.
- Circulating TSH levels are known to show a circadian rhythm & diurnal variation. The diagnosis based on one TSH value which fluctuates is not reliable. Clinical correlation is mandatory.
- Values <0.03 uIU/mL need to be clinically correlated due to presence of a rare TSH variant in some individuals.

Remarks: Please correlate results clinically.

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Iron Profile

Serum Sample

Accession No: 0346832SST Collected On: 29-Jan-26 08:56 Received On: 29-Jan-26 12:21 Approved On: 29-Jan-26 15:14

Observation	Result	Unit	Biological Ref. Interval	Method
Iron	34	µg/dL	49-181	Pyridylazo Dye
Total Iron Binding Capacity	292	µg/dL	261 - 462	Chromazurol B
Transferrin Saturation	11.64	%	19 - 39	Calculated

Remarks: Please correlate results clinically.



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Conditions Of Reporting

- ▶ The report results are for information and interpretation for your referring doctor. Reports are to be correlated with the patient's clinical history.
- ▶ Biological Reference Range/Interval is suggested for your Gender and Age on the basis of available literature. All reference ranges are to be reconsidered by physician's advice for your specific care.
- ▶ This Medical Report is a professional opinion, not a diagnosis.
- ▶ The report will carry the name and age provided at the time of registration. To maintain confidentiality, certain reports may not be e-mailed at the discretion of the management.
- ▶ All the notes and interpretation beneath the pathology result in the report provided are for educational purpose only. It is not intended to be a substitute for physician's consultation.
- ▶ Results of tests may vary from laboratory to laboratory and in some parameters from time to time for the same patients. Test results and reference range may also vary depending on the technology and methodology used. Laboratory test results may also vary depending on the age, sex, time of the day sample has been taken, diet, medication and limitation of modern technology.
- ▶ In case of any unexpected or alarming test results, please contact us immediately for re-confirmation, further discussion, clarifications and rectifications, if needed.
- ▶ In case of any discrepancy due to typing error, kindly get it rectified immediately.
- ▶ Neither HOD or its employees/representatives assume any liability or responsibility for any loss or damage that may be incurred by any person as a result of interpreting the meaning of this report.
- ▶ Test results are not valid for medico legal purposes.
- ▶ In case of any issues or suggestions about your test results, please email us on quality@houseofdiagnostics.com
- ▶ The courts (forums) at Delhi shall have exclusive jurisdiction in all disputes/claims concerning the tests and the results of the tests. Our liability is limited to the amount of investigations booked with us.

DOC#COR20200707

Facilities Available

Radiology

- ▶ 3T MRI & 1.5T MRI
- ▶ CT Scan
- ▶ Digital X-Ray
- ▶ Mammography
- ▶ Open / Standing MRI
- ▶ Bone DEXA Scan

Pathology

- ▶ Biochemistry
- ▶ Immunoassay
- ▶ Hematology
- ▶ Clinical Pathology
- ▶ Serology
- ▶ Microbiology

Nuclear Medicine

- ▶ **India's First** Simultaneous PET-MRI
- ▶ Whole Body PET/CT Scan
- ▶ DTPA / DMSA Renal Scans
- ▶ Thyroid Scan
- ▶ Whole Body Bone Scan
- ▶ HIDA Scan • Rest MUGA

Cardiology Investigations

- ▶ ECG (Electrocardiogram)
- ▶ Echocardiography
- ▶ TMT
- ▶ Stress Echocardiography
- ▶ Stress Thallium

Neurology Investigations

- ▶ EEG - ElectroEncephaloGram
- ▶ EMG - ElectroMyoGraphy
- ▶ NCV - Nerve Conduction Velocity
- ▶ VEP - Visual Evoked Response
- ▶ SSEP

Dental Imaging

- ▶ CBCT - Cone Beam CT Scan
- ▶ OPG - OrthoPantomoGram

Other Tests

- ▶ PFT